



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
980 9th Street, Suite 500
Sacramento, CA 95814
Phone: 916-324-8176 | Fax: 916-255-5241
www.DMHC.ca.gov

March 19, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with O'Connor Hospital
Block Transfer Filing No. 2026-HOSP-277
Filed on March 18, 2026

Dear Karen Choate:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **O'Connor Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Please confirm the Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

Plan Response1: The Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

2. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Plan Response2: The Plan will not transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

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3. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Plan Response3: The hospital termination is part of a health system termination; all four hospitals are under a single contract.

4. Please confirm the Plan has zero medical groups which routinely and non-routinely utilize the Terminating Hospital.

Plan Response4: No (0) medical groups routinely and non-routinely utilized the Terminating Hospital.

5. Please confirm whether the contract termination between the Plan and O'Connor Hospital impacts Point of Service (POS) enrollees.

Plan Response5: The contract termination between the Plan and the Terminating Provider impacts Point of Service (POS) enrollees.

6. The Plan indicates that Cancer services are available at the Terminating Hospital. Please specify which Cancer services are available at the Terminating Hospital and confirm which Alternate Hospital(s) provides each service(s).

Plan Response6: The historical OSHPD/HCAI annual disclosure dataset shows a positive (1) indicator for the Oncology service line at the Terminating Hospital. For Alternate Hospital(s) where the OSHPD/HCAI dataset did not clearly indicate Oncology, the Plan also reviewed facility websites and applied the same indicator-level approach (oncology services offered: Yes/No) to confirm oncology service availability at El Camino, Good Samaritan, Washington, etc. The Plan relied on OSHPD/HCAI where definitive and used website review as supplemental confirmation to support service-line level oncology availability, recognizing that "Oncology" is a broad category and that website content is not standardized or comprehensive.

7. The Plan did not indicate what services are offered at the Terminating Hospital. Please confirm if the following services are available:
 - a. Abortion Services
 - b. Acute Neonatal
 - c. Acute Pediatric
 - d. Acute Psychiatric
 - e. Burn Services
 - f. Coronary ICU

- g. Heart Catheterization
- h. Hemodialysis
- i. Labor Delivery Room
- j. Neonatal ICU
- k. Neuro Surgery
- l. Open Heart Surgery
- m. Pediatric ICU
- n. Petscan
- o. Radiation Therapy

Plan Response7: As the terminating provider did not report its current service inventory to HCID (formerly OSHPD), DMHC's web portal also did not populate which services are offered. The services offered below is based on historical data the provider reported to OSHPD (Y = Yes & N=No):

Service	Offered	Service	Offered
a. Abortion Services	N	i. Labor Delivery Room	Y
b. Acute Neonatal	Y	j. Neonatal ICU	Y
c. Acute Pediatric	Y	k. Neuro Surgery	Y
d. Acute Psychiatric	N	l. Open Heart Surgery	Y
e. Burn Services	N	m. Pediatric ICU	N
f. Coronary ICU	Y	n. Petscan	Y
g. Heart Catheterization	Y	o. Radiation Therapy	Y
h. Hemodialysis	Y		

8. The Plan's PPO line of business is licensed by the Department. Please confirm the following:
- a. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

Plan Response8: The Plan will send Department-approved Enrollee Transfer Notices to all PPO enrollees residing within a 15-mile radius of the Terminating Hospital.

9. For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:
- a. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 - 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.

- ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.

Plan Response9: No provider groups primarily utilize the Terminating Hospital.

10. For each Alternate Hospital, please summarize or confirm the following:

- a. The Plan contacted the hospital regarding the Block Transfer,
- b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer

Plan Response10: The Plan outreaches to the hospitals with occupancy rates at 80% or higher and documented any concerns and capacity response(s) in the filing portal. For hospitals with occupancy rates <80%, justification is not provided as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees per §1300.67.1.3(2)(L): *"Based upon the data made public on the Office of Statewide Health Planning and Development's website, for each of the proposed Alternate Hospitals, provide the available bed occupancy rate for the most recently completed calendar year. If the rate is at 80% or higher, please provide justification as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees."* All alternate hospitals are <80% capacity.

11. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

Plan Response11: Please refer to Plan Response4.

12. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals:

a. Any out-patient or in-patient services

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

Plan Response12: The Plan does not anticipate any problems with the ability of the Alternate Hospitals to provide the same services available at the Terminating Hospital. If there is a service identified that is a covered benefit and only available at the terminating hospital the Plan will make every effort to ensure services are provided at the terminating hospital. When a covered service is not available or accessible at a participating network hospital or specialty facility in the Plan's' network, members will be allowed to receive that service at a non-participating hospital as appropriate, reimbursed at in-network benefit levels.

13. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Plan Response13: To date, there are no enrollees who have a procedure scheduled and authorized by the Plan at the Terminating post-termination.

14. Please affirm that the Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

Plan Response14: The Plan affirms that Alternate Hospitals for the Plan's Narrow Networks meet the Knox-Keene Act's geographic access standards and/or Department approved alternative access standards.

Please submit the Plan's response by **March 26, 2026** via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,



Christopher Loyd
Supervisor I
Office of Plan Monitoring

Note to Reviewer: Below are the responses to the additional questions received on March 19th:

